Lab 6

Case Study:
A Cluster on the Surgical Ward

Day 2 · 13:15–15:00 · 105 min
Groups of 4

LO3–LO5: combine genomic and epidemiological evidence to decide whether an outbreak is happening, and communicate the decision

Open your handout

Lab 6 handout

`Lab6_ssi_outbreak_case_study.pdf`

- **Printed:** Lab 6 in your handout pack
- **PDF:** the course folder (link from the instructor) or the USB stick

> The handout has every step. Copy reads, URLs and commands from the **PDF**, not from this screen.

About this case

- The **genomes are real**: MRSA isolates from a published outbreak investigation
- The **hospitals, wards, patients and dates are fictional**, so the case feels like our setting

> The real story is revealed at the end.

THE SITUATION

District Hospital A · Monday, 16 March 2026

The IPC nurse writes to you:

> *"Since early February we've had **more MRSA wound infections than usual on Surgical Ward B**. The surgeons say it's bad luck. Theatre staff say it's the ward. The ward says it's the theatre.*

The question

> *The director asks whether we should **close Ward B**. That would cancel about 40 operations a week. We got genomes back last week for all MRSA isolates since September.*
>
> ***Can you tell us what's going on by Wednesday's IPC meeting?"***

Your evidence

1. **Microreact project** from Lab 5: tree + timeline + metadata
2. **SNP matrix:** `koser_snp_matrix.tsv` (opens in Excel)
3. **Pathogenwatch results** from Lab 5: ST and AMR
4. **Metadata:** `lab6_metadata.csv`

Group roles

Role	Job
Genomics lead	Reads the tree and SNP matrix
Epidemiologist	Builds the timeline: who was where, when
Clinician / IPC	Thinks through transmission routes and actions
Communicator	Writes and gives the 5-minute briefing

Plan for this lab

Part	What	Time
A	Describe the cases	15 min
B	What does the genome say?	25 min
C	Genomics + epidemiology together	20 min
D	Prepare the briefing	25 min
E	Briefings to the IPC committee	15 min

Describe the cases

Sketch an **epidemic curve**: weeks across, number of cases up.
Two colours: **Ward B** and **other wards / hospitals**.

QUESTIONS

1. How many MRSA isolates in total? How many from Ward B since 1 February?
2. Do all Ward B cases look the same on the **lab report**? Does that make them one outbreak?

What does the genome say?

1. **Sequence types:** group the isolates by ST (Pathogenwatch)
2. **SNP distances:** highlight all pairs **≤ 15 SNPs** apart
3. Which isolates form a **cluster**, all close to each other?
4. **Tree:** colour by **ward**, then by **theatre**. Is the cluster a tight group on short branches?

≤ 15 SNPs: a common working threshold for possible recent *S. aureus* transmission

Put genomics and epidemiology together

For **each** Ward B case since February:

Patient	In the cluster?	Ward / theatre / time link?	Linked · not linked · uncertain

Use the full table in your handout.

PART C

Discuss in your group

1. Is there an outbreak? How confident are you?
2. Is a Ward B patient **outside** the cluster, though on the ward at the same time?
3. Is a cluster patient **not** on Ward B? What do they share? → **likely route?**
4. Are isolates from **other hospitals** in the cluster?
5. What **other information** would you want?

Prepare a 5-minute briefing

For the IPC committee: director, chief surgeon, nursing lead, lab head. **Not genomics experts.**

RULES

- One flip-chart page or 3 slides
- Explain any jargon: “SNP” → “genetic differences”
- **No more than one figure**

PART D

Briefing template

1. **Bottom line:** “We found / did not find evidence of an outbreak of ___ linked to ____.”
2. **What we did:** “We sequenced __ MRSA isolates from __ to __.”
3. **What we found:** one visual + 2–3 bullets
4. **What it means:** which cases are linked; the likely route
5. **Recommendations:** 3 actions, with who and by when
6. **What we don’t know yet**

Briefings to the IPC committee

Each group has **5 minutes**. The committee must ask at least one question:

“Should we close Ward B?”

“Is it the surgeons' fault?”

“How sure are you?”

“What will this cost?”

Pause

Instructor: don’t go to the next slide until every group has given its briefing

The reveal

The genomes come from **Köser et al., *NEJM* 2012**: a suspected MRSA outbreak in a **neonatal unit** in Cambridge, UK.

- Sequencing separated the **outbreak isolates** from unrelated MRSA that looked identical on routine tests
- A follow-up study (**Harris et al. 2013**) traced transmission into the community and found a **staff carrier**, which ended the outbreak

Genomic data: ENA project PRJEB2912 (Köser et al. 2012). Scenario metadata are fictional and created for teaching.

Take-home messages

1. **Same species + same antibiogram ≠ same outbreak**
2. Genomics finds **hidden links**: shared theatre, staff, equipment
3. Genomics can **prevent unnecessary actions**, like closing a ward
4. Genomics is only useful with **good metadata** and **clear communication**